

C) Automated RPM Weekly Intelligence Summary

Window: 2024-05-01 → 2024-06-30 | Hourly

Triage: Red (MD review recommended)

Signal	Count (hours)
Brady hours	20
Severe brady hours	26
Tachycardia hours	7
Tachypnea hours	14
Low-confidence hours	29

Top episodes to review

Flag	Window	Hours	Suggested trigger
Tachypnea (RR avg >24)	2024-06-24 00:00→2024-06-24 09:00	10	If SpO ₂ low or concurrent HR rise → MD/Resp
Severe bradycardia (HR min <45)	2024-06-23 21:00→2024-06-24 05:00	9	If symptomatic or HR<45 sustained → MD/Cardiology
Bradycardia (HR avg <50)	2024-06-23 22:00→2024-06-24 05:00	8	If persistent/new → MD; review meds
Tachycardia (HR avg >100)	2024-06-27 20:00→2024-06-28 02:00	7	If sustained >2h or HR>120 → MD
Severe bradycardia (HR min <45)	2024-06-25 01:00→2024-06-25 05:00	5	If symptomatic or HR<45 sustained → MD/Cardiology
Bradycardia (HR avg <50)	2024-06-24 07:00→2024-06-24 11:00	5	If persistent/new → MD; review meds
Bradycardia (HR avg <50)	2024-06-25 02:00→2024-06-25 05:00	4	If persistent/new → MD; review meds
Severe bradycardia (HR min <45)	2024-06-24 07:00→2024-06-24 10:00	4	If symptomatic or HR<45 sustained → MD/Cardiology